INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Abiraterone and prednisolone: metastatic hormone-sensitive prostate cancer

Understand your treatment. Know what to expect. Keep your plan to hand.

Treatment pattern
Ongoing
Follow the prescribed daily plan

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00577
Version 3

> Daily tablets plus ongoing ADT

Treatment stages

01 **Continuous oral treatment**
Daily tablets plus ongoing ADT

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Newly diagnosed high-risk metastatic hormone-sensitive prostate cancer

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology urgently for sudden illness, persistent vomiting, inability to drink, jaundice, new breathlessness or severe rash. Call 112/999 for severe chest pain, severe breathing difficulty, collapse, seizure, or new leg weakness with loss of bladder or bowel control.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

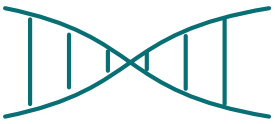
How the medicine works

This treatment aims to control prostate cancer and delay progression. The expected benefit depends on your disease setting, previous treatment and general health.

ANDROGEN PATHWAY TREATMENT

Abiraterone

Oral targeted treatment



Adds deeper androgen suppression to ADT

ADT lowers testosterone from the testes. Abiraterone blocks CYP17 to reduce androgen production elsewhere too. Prednisolone limits hormonal side effects. This combination can be offered early, before ADT stops controlling the cancer.

ADT lowers testosterone → Abiraterone reduces more androgen → Cancer growth signals reduced

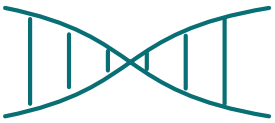
What to watch for

Never stop prednisolone suddenly or when abiraterone is held unless the team gives a steroid plan. Ask about a steroid alert card and illness instructions. Contact the team if vomiting prevents you keeping tablets down. Abiraterone with prednisolone must not be combined with radium-223.

CORTICOSTEROID

Prednisolone

Prescribed oral steroid



Reduces excess hormonal drive

Prednisolone suppresses ACTH drive during abiraterone treatment, helping control blood pressure, fluid retention and potassium. It needs its own illness and withdrawal plan.

ACTH drive reduced → Hormonal side effects limited → Abiraterone supported

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

What the evidence can tell you

Research supports a treatment discussion; it cannot predict one person's outcome.

In the LATITUDE trial's 2017 analysis of newly diagnosed high-risk metastatic hormone-sensitive cancer, 66% receiving abiraterone plus prednisone and ADT were alive at 3 years versus 49% receiving ADT and placebos (hazard ratio 0.62; 95% confidence interval 0.51–0.76). The difference is a trial average, not an individual forecast.

CHAPTER 02

Your schedule, tests and appointments

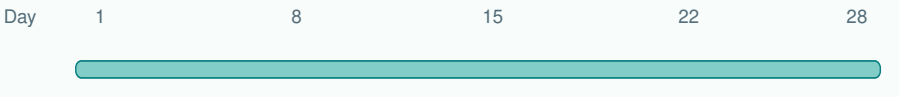
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment course

Abiraterone and prednisolone: metastatic hormone-sensitive prostate cancer

28-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

DAILY TABLETS PLUS ONGOING ADT

Continuous oral treatment

Continue while helping and tolerated

Abiraterone 1000 mg once daily plus prednisolone 5 mg once daily, continuously while helping and tolerated. Continue your ADT.

Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.

BASELINE → TREATMENT → FOLLOW-UP

Your tests and treatment checks

Use the individual prescription and appointment calendar. No universal tumour-marker panel or scan interval is assumed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00577 v3: official regimen

Tests in your plan

Baseline bloods and tests

WHEN

Full blood count, kidney and liver tests, glucose and blood pressure; ECG if clinically indicated or if you have a cardiac history. Review potassium, swelling, heart health and all medicines.

Checks organ safety and suitability before treatment.

Treatment safety monitoring

WHEN

Blood count, kidney and liver tests, glucose and blood pressure every 2 weeks for the first 3 treatment months, then every 4 weeks. Potassium may need closer checks if low; assess swelling and cardiac symptoms. ECG if clinically indicated or with cardiac history.

Finds changes that may need supportive care, a dose hold or adjustment.

Specialist and cancer tests

If part of your plan

WHEN

PSA and testosterone help assess cancer response and ADT suppression. High risk here requires at least two of Gleason score 8 or more, at least three bone lesions and measurable spread to an internal organ; your team explains which criteria apply.

Distinguishes treatment selection, hormone monitoring and cancer response.

THE WHOLE TREATMENT JOURNEY

Abiraterone and prednisolone: metastatic hormone-sensitive prostate cancer: treatment and checks

Newly diagnosed prostate cancer that has spread, remains hormone-sensitive and meets the NCCP high-risk definition, treated together with ADT.

Abiraterone 1000 mg once daily plus prednisolone 5 mg once daily, continuously while helping and tolerated. Continue your ADT.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Blood and organ safety NCCP	Full blood count, kidney and liver tests, glucose and blood pressure; ECG if clinically indicated or if you have a cardiac history. Review potassium, swelling, heart health and all medicines.	Blood count, kidney and liver tests, glucose and blood pressure every 2 weeks for the first 3 treatment months, then every 4 weeks. Potassium may need closer checks if low; assess swelling and cardiac symptoms. ECG if clinically indicated or with cardiac history.	Extra tests may be needed for symptoms, dehydration or abnormal results.	Review results, recovery and previous toxicity before any restart or next dose.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.

Care / check	Before starting	Early treatment	Ongoing checks	Dose hold / restart	After treatment
Appointments and medicines NCCP	Agree the treatment aim, dose and complete medicines list.	Take abiraterone on an empty stomach: at least 2 hours after food, then wait at least 1 hour before eating. Swallow tablets whole with water. Take prednisolone with food. If a daily dose of either medicine is missed, take the usual dose the next day; do not double it.	Keep a current dose and symptom record.	Never stop prednisolone suddenly or when abiraterone is held unless the team gives a steroid plan. Ask about a steroid alert card and illness instructions. Contact the team if vomiting prevents you keeping tablets down. Abiraterone with prednisolone must not be combined with radium-223.	Keep follow-up and recovery checks even after the last dose.
Cancer-specific and specialist tests NCCP	PSA and testosterone help assess cancer response and ADT suppression. High risk here requires at least two of Gleason score 8 or more, at least three bone lesions and measurable spread to an internal organ; your team explains which criteria apply.	Confirm the booked test dates with your team.	Safety tests and response tests have different purposes.	Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval.	Agree the named follow-up team and remaining checks.

What each check is for

1



Treatment and review

Daily tablets plus ongoing ADT

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Ask / **act:** Blood count, kidney and liver tests, glucose and blood pressure every 2 weeks for the first 3 treatment months, then every 4 weeks. Potassium may need closer checks if low; assess swelling and cardiac symptoms. ECG if clinically indicated or with cardiac history.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.



STEP 1

Before starting

Full blood count, kidney and liver tests, glucose and blood pressure; ECG if clinically indicated or if you have a cardiac history. Review potassium, swelling, heart health and all medicines.

STEP 2

Taking or receiving treatment

Take abiraterone on an empty stomach: at least 2 hours after food, then wait at least 1 hour before eating. Swallow tablets whole with water. Take prednisolone with food. If a daily dose of either medicine is missed, take the usual dose the next day; do not double it.

STEP 3

Tests and reviews

Blood count, kidney and liver tests, glucose and blood pressure every 2 weeks for the first 3 treatment months, then every 4 weeks. Potassium may need closer checks if low; assess swelling and cardiac symptoms. ECG if clinically indicated or with cardiac history.

STEP 5

Leaving with a clear plan

Confirm the next appointment, dose, support medicines and verified daytime and out-of-hours phone numbers.

Your exact treatment schedule

Tests and preparation

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Your continuous-treatment review window.

Timing is a general guide; symptoms may start or stop at other times.

Treatment or check	Timing in this view
Abiraterone + prednisolone every day	 **Days 1–28** Abiraterone 1000 mg once daily plus prednisolone 5 mg once daily, continuously while helping and tolerated. Continue your ADT.

This 28-day window is a review calendar, not a rest cycle or a fixed treatment duration.

Treatment continues daily across review dates unless your team tells you to hold or stop. No automatic rest week is scheduled.

> Use your treating service's written temperature threshold and urgent-contact plan. Fever, shivering or sudden illness can be serious even without a high temperature. Do not wait for the next visit. Severe chest pain, severe breathing difficulty, collapse or a seizure needs emergency help: call 112 or 999.

Read the whole review window as text

Day 1–28: Every day, including weekends

Take abiraterone on an empty stomach: at least 2 hours after food, then wait at least 1 hour before eating. Swallow tablets whole with water. Take prednisolone with food. If a daily dose of either medicine is missed, take the usual dose the next day; do not double it. Follow your prescription. A hold or restart must be agreed with the team.

Support around the medicines

1 Preventing and treating sickness

This is minimal-to-low oral sickness risk; the team agrees rescue medicine if needed.

2 Medicine-specific support

3 Living well during hormone treatment

Discuss hot flushes, fatigue, mood, sexual function and fertility. Ask for an individual activity and bone-health plan, including appropriate strength exercise, falls prevention, calcium intake and vitamin D advice. These treatments can change glucose, cholesterol and muscle strength.

4

Medicine and interaction review

Bring all prescription, non-prescription and herbal medicines. Check with the oncology pharmacist before starting or stopping anything.

Your diagnosis and treatment setting

Specialist tests

PSA and testosterone help assess cancer response and ADT suppression. High risk here requires at least two of Gleason score 8 or more, at least three bone lesions and measurable spread to an internal organ; your team explains which criteria apply.

Why ADT continues

Androgen deprivation therapy (ADT) lowers testosterone from the testes. This medicine adds another form of hormone-pathway blockade. Even when cancer becomes castration-resistant, continued testosterone suppression remains part of this treatment unless you have had surgical castration.

Treatment aims and choices

Deciding together

This treatment aims to slow cancer growth and help control symptoms. Your team explains the expected benefit, uncertainty and alternatives for your situation.

Review and alternatives

Continue your prescribed androgen deprivation therapy (ADT), unless you have had surgery to remove both testes. The team reviews symptoms, blood tests, PSA and scans according to your own plan; there is no universal scan interval. Discuss suitable alternatives, supportive care and the option of declining treatment.

Where radiotherapy fits

Radiotherapy may treat the prostate, a recurrence or a troublesome site such as a painful bone lesion. Its purpose depends on your disease setting. Ask the team whether it belongs in your plan; starting these tablets does not itself schedule or replace radiotherapy.

Questions worth asking

undefined

undefined

undefined

undefined

undefined

undefined

undefined

undefined

> **Try explaining the plan back:** Explain when you take or receive treatment.,Name the tests and next review date.,Show how to contact the team urgently.,Explain what to do if a dose is missed or treatment is held.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous oral treatment

Active medicines: Abiraterone + Prednisolone

Emetogenic risk: Minimal to low (oral)

Medicine	Dose / route	When
Antiemetic plan for current oral dose	Individual prescription	As agreed with oncology/pharmacy Review QT prolongation and all interactions; no automatic intravenous (IV) chemotherapy script.

Additional phase-specific support

- Take abiraterone on an empty stomach: at least 2 hours after food, then wait at least 1 hour before eating. Swallow tablets whole with water. Take prednisolone with food. If a daily dose of either medicine is missed, take the usual dose the next day; do not double it.
- Blood count, kidney and liver tests, glucose and blood pressure every 2 weeks for the first 3 treatment months, then every 4 weeks. Potassium may need closer checks if low; assess swelling and cardiac symptoms. electrocardiogram (ECG) if clinically indicated or with cardiac history.
- Never stop prednisolone suddenly or when abiraterone is held unless the team gives a steroid plan. Ask about a steroid alert card and illness instructions. Contact the team if vomiting prevents you keeping tablets down. Abiraterone with prednisolone must not be combined with radium-223.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Energy, balance and thinking

Abiraterone + prednisolone

Report troublesome fatigue, falls or confusion. Seizure, collapse or sudden neurological symptoms need emergency help.

3 Heart and blood pressure

Abiraterone + prednisolone

Keep blood-pressure checks. Chest pain, fainting or severe breathlessness needs urgent help; report ankle swelling or palpitations.

5 Appetite and bowels

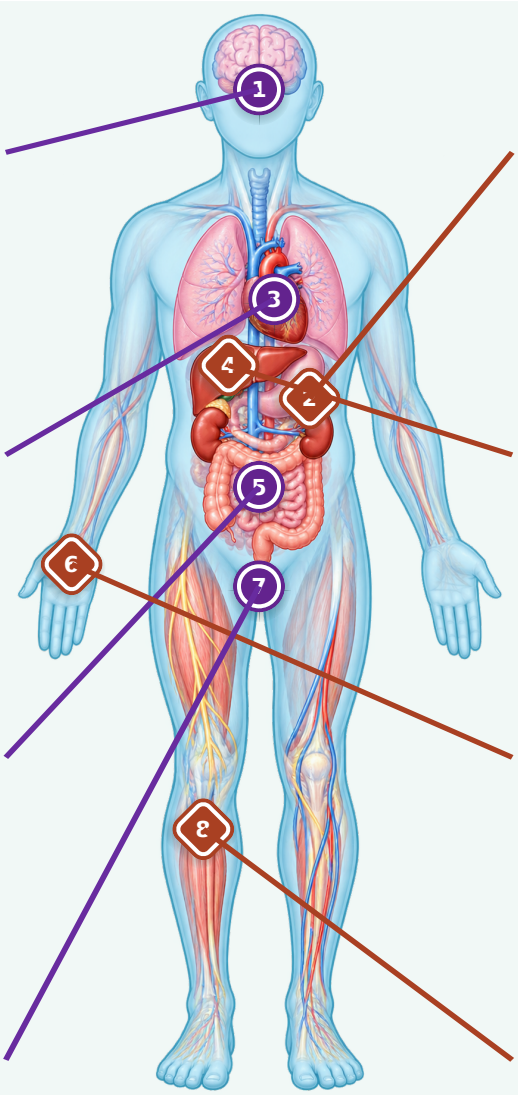
Abiraterone + prednisolone

Report persistent vomiting or diarrhoea, inability to drink or take medicines, and unintended weight loss.

7 Sexual health and fertility

ADT and Abiraterone + prednisolone

Discuss erections, libido, fertility and contraception. Avoid exposing a pregnant partner to these medicines; follow the product-specific precautions.



2 Steroid and potassium balance

Abiraterone / prednisolone

Keep prednisolone as prescribed. Do not stop abruptly. Low potassium may cause weakness or palpitations; vomiting or serious illness needs a steroid plan.

4 Liver and blood tests

Abiraterone + prednisolone

Keep liver blood tests. Report yellow skin or eyes, dark urine, reduced appetite or a major change in wellbeing.

6 Skin changes

Abiraterone + prednisolone

Report a new rash. Severe rash with blisters, peeling, fever or mouth/eye sores needs urgent assessment.

8 Bones and muscle strength

ADT and Abiraterone + prednisolone

Discuss bone-density assessment, falls prevention, suitable resistance and aerobic activity, calcium intake and vitamin D needs.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology urgently for sudden illness, persistent vomiting, inability to drink, jaundice, new breathlessness or severe rash. Call 112/999 for severe chest pain, severe breathing difficulty, collapse, seizure, or new leg weakness with loss of bladder or bowel control.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00577 v3: official regimen**
   https://healthservice.hse.ie/documents/6393/577_v3_Abiraterone_and_prednisoLONE_Therapy.pdf
2. **Zytiga: EMA product information, including trial evidence**
   https://www.ema.europa.eu/en/documents/product-information/zytiga-epar-product-information_en.pdf
3. **HSE: cancer treatment side effects**
   https://www.hse.ie/conditions/cancer/if-you-have-side-effects/
4. **NCCP supportive-care antiemetics for NCIS**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
5. **NCI: hormone therapy for prostate cancer**
   https://www.cancer.gov/types/prostate/prostate-hormone-therapy-fact-sheet